

Lichen Sclerosus (LS)

Patient Information · A long-term skin condition of the genital area, for people of any sex

WARWIKI

Lichen sclerosus (LS) is a long-term skin condition that most often affects the genital and anal skin. It causes itching, soreness, white patches and scarring. It is **not an infection** and cannot be passed to a partner. A strong steroid ointment, used the right way and kept up over time, **controls it for most people and helps prevent scarring**.

Signs of LS

- **Any skin:** itching, soreness, small splits, bruising, and white, thin or shiny patches.
- **Vulva:** the inner lips can shrink or fuse, the hood can scar over the clitoris, and the vaginal opening can narrow. Sex may hurt.
- **Penis:** a tight foreskin, painful erections, white patches on the tip, and a narrow urine opening that makes the stream spray or slow. In about 1 in 5 men with LS, it also scars the urethra.

Diagnosis is usually made by looking at the skin. A small skin sample (biopsy) is taken if anything looks unusual or does not heal.

Benefits and Risks in Numbers

over
9 in 10

who kept up their ointment had symptoms controlled, vs about 6 in 10 who did not

3 in 100

who kept it up got new scarring, vs 40 in 100 who did not

None

who kept it up got skin cancer or precancer, vs about 5 in 100 who did not

From a 5-year study of 507 women with vulvar LS. Your own risk may differ; ask your team.

How to Use Your Steroid Ointment

- 1 **At first:** usually once a day for 1–3 months, until the skin calms down.
 - 2 **Then:** less often, such as once or twice a week, to keep LS quiet. Many people need this for years.
 - 3 **How much:** a thin layer, rubbed in, only on the affected skin. A little goes a long way: a 30 g tube usually lasts 6 months or longer.
- Ask your team to show you, with a mirror, **exactly where** to apply it.
 - The pack may warn against use on genital skin. Used as your team directs, it is safe here.
 - Estrogen or testosterone creams do not treat LS.

Learn the Terms

Clobetasol

A strong steroid ointment, the usual first treatment for LS.

Ointment

A greasy base. It stings less than a cream on sore skin.

Emollient

A plain, unscented moisturizer or soap substitute that protects the skin.

Maintenance

Using the ointment less often, long term, to keep LS quiet.

Phimosis

A foreskin too tight to pull back.

Meatal stenosis

A narrowed urine opening at the tip of the penis.

Biopsy

A small skin sample checked under a microscope.

Is LS linked to cancer? LS raises the chance of skin cancer of the vulva or penis. The risk is low, but higher than in people without LS. Keeping LS under control with ointment is linked with a lower risk. Check your skin regularly and keep your follow-up visits.

First 1–3 months

Ointment daily. A visit to check the response.

After that

Ointment once or twice a week, or as your team advises.

Every 6–12 months

A skin check, for life, even when you feel well.

Any time

See your team for a new lump, sore or change.

Check Your Skin

Look at your skin about every 3 months, and any time symptoms change. Use good light and a hand mirror.

- 1 Look** over the whole area, including the skin around the anus. If you have a foreskin, pull it back if you can.
- 2 Feel** gently for lumps, thick or rough patches, or tender spots.
- 3 Note** any sore that does not heal, bleeding, or a change in color. Report it.

Everyday Skin Care

- Wash with water or a soap substitute instead of soap. Avoid scented products and wipes.
- Dry gently after urinating. A plain barrier ointment such as petroleum jelly protects the skin from urine.
- Use a lubricant for sex. If you smoke, stopping lowers the risk of cancer of the penis.

When Scarring Needs Surgery

- **Tight foreskin:** circumcision can cure LS that affects only the foreskin.
- **Narrow urine opening or urethra:** the opening can be widened. A scarred urethra is repaired with a graft from the cheek, because genital skin used as a graft tends to scar again.
- **Vulva:** scar bands that narrow the opening or trap the clitoris can be released.
- Keep using your ointment before and after surgery. Surgery treats scarring; the ointment treats LS.

Call your care team if you have:

- Itching or soreness that comes back despite the ointment
- A new lump, thick patch, sore that does not heal, bleeding, or color change
- A spraying or weak urine stream, or pain with sex
- After LS surgery: a temperature of **100.4 °F (38 °C)** or higher

Go to an emergency department if:

- You cannot urinate and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

- 1.** LS is long term, but a steroid ointment controls it for most people.
- 2.** Keep using the ointment as advised, even when the skin looks better.
- 3.** Check your skin and keep yearly visits. Report any lump or sore that does not heal.